Translating Evidence Into Design:

Equity-Informed Development of a Digital Mental Health Intervention

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The Gap We Designed Around

According to the 2024 National Survey of Children's Health[1], low-income teens (ages 12 to 17) are 1.8 times as likely to report needing mental health care and not getting it.

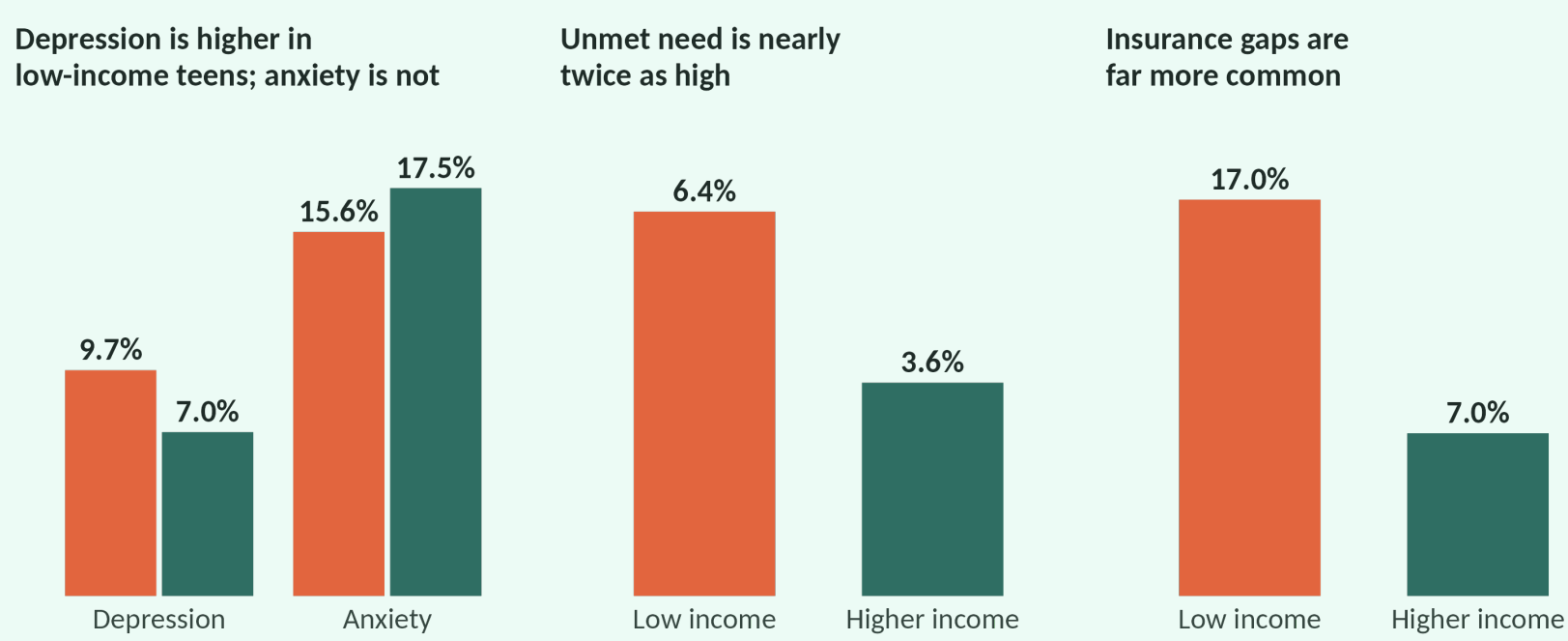
Why this matters

- Low-income teens face more barriers to mental health care
- Cost and insurance gaps can block in-person treatment
- A digital tool can help, if built around those barriers



What we found

2024 National Survey of Children's Health, ages 12 to 17 (n = 21,960). Survey-weighted in R. Low income = at or below 200% of the poverty level.





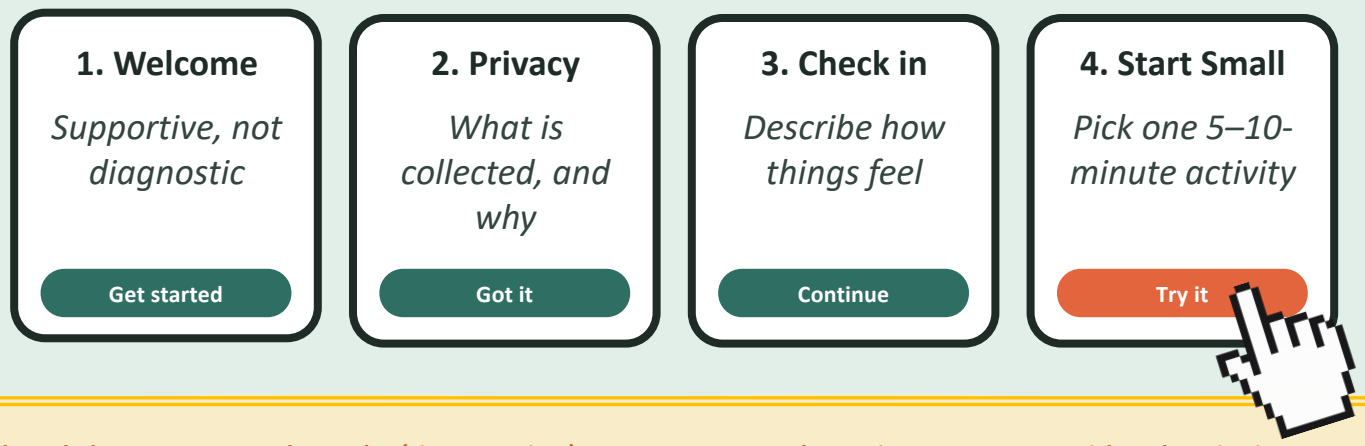
From findings to design

Higher Depression Rates → Starter module for low mood & motivation

Nearly Double the Unmet Need → Self-guided, with route to a trusted adult

More insurance gaps → No insurance details needed to start

The prototype onboarding flow



Going Forward

- Test with teens and caregivers
- Add a feasibility evaluation
- Plan school and community rollout

Takeaway: Build for access, not just diagnosis!

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1. *2024 National Survey of Children's Health (HRSA MCHB; U.S. Census Bureau).*